Hospital Patient Pathway

Operational BPMN - vector PDF review edition

Source: editable 1-17.bpmn | Camunda 8.9 | submission snapshot 29 September 2026

The next page contains the complete collaboration. Pages after it isolate each of the 17 hospital stages so task names, gateways, lanes and flows can be read at normal PDF zoom. All diagrams are vector based. Use the PDF bookmarks to jump to a stage. The editable BPMN is the authoritative model.

System boundary and participants

Executable participant: Hospital Patient Pathway System. Hospital activities are divided among 11 lanes: medical secretaries, consultants, clinical nurse specialists, outpatient bookings, treatment bookings, finance, pathway coordinators, call handlers, administrative management, IT support and senior management.

External collaboration participants: referring organisation, patient, scheduling service, treatment/laboratory/imaging service, insurer/funder, payment service provider, external correspondence service and delay-tracking system. Dashed message flows in the complete collaboration identify cross-boundary communication. These external pools are non-executable in the local hospital deployment; Java simulation supplies test replies.

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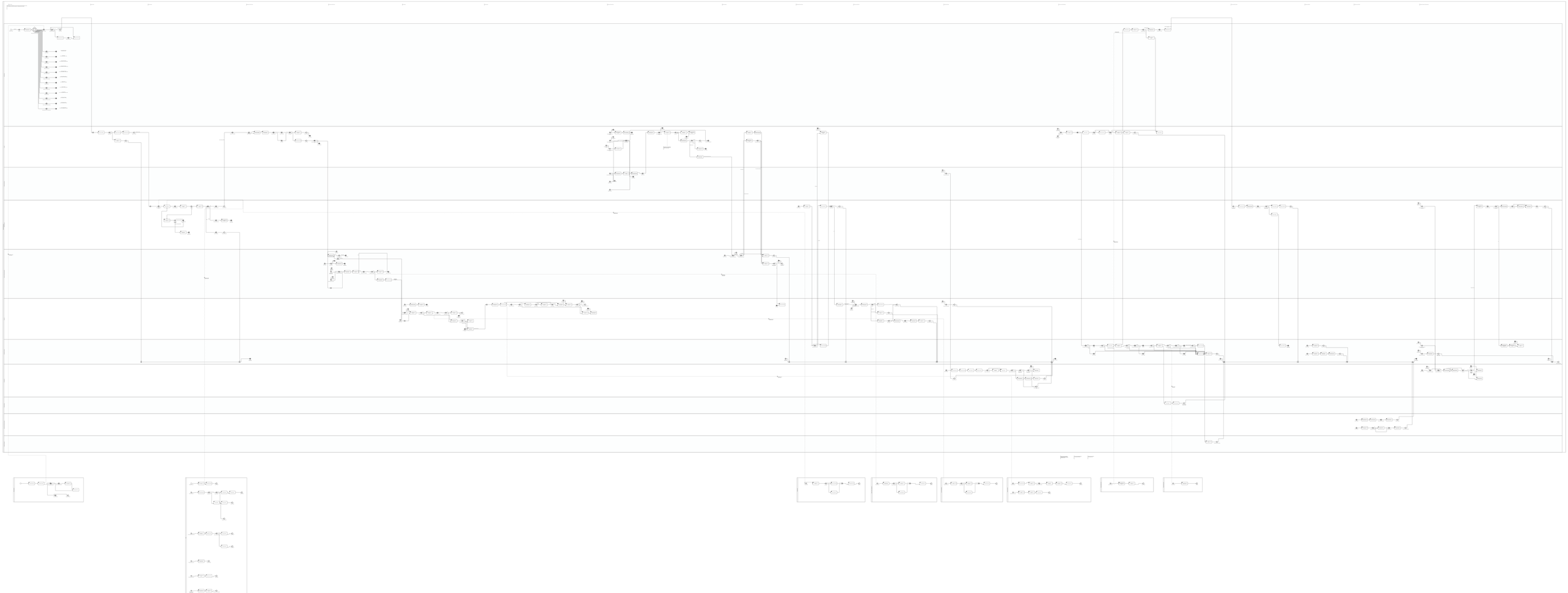
Validation and limits

Camunda Modeler lint and hospital deployment-scope engine checks report 0 errors and 0 warnings. Runtime evidence covers a full synthetic main path and selected alternatives. It does not prove live third-party integration or every timer and concurrent branch. See submission/TEST_RESULTS.md.

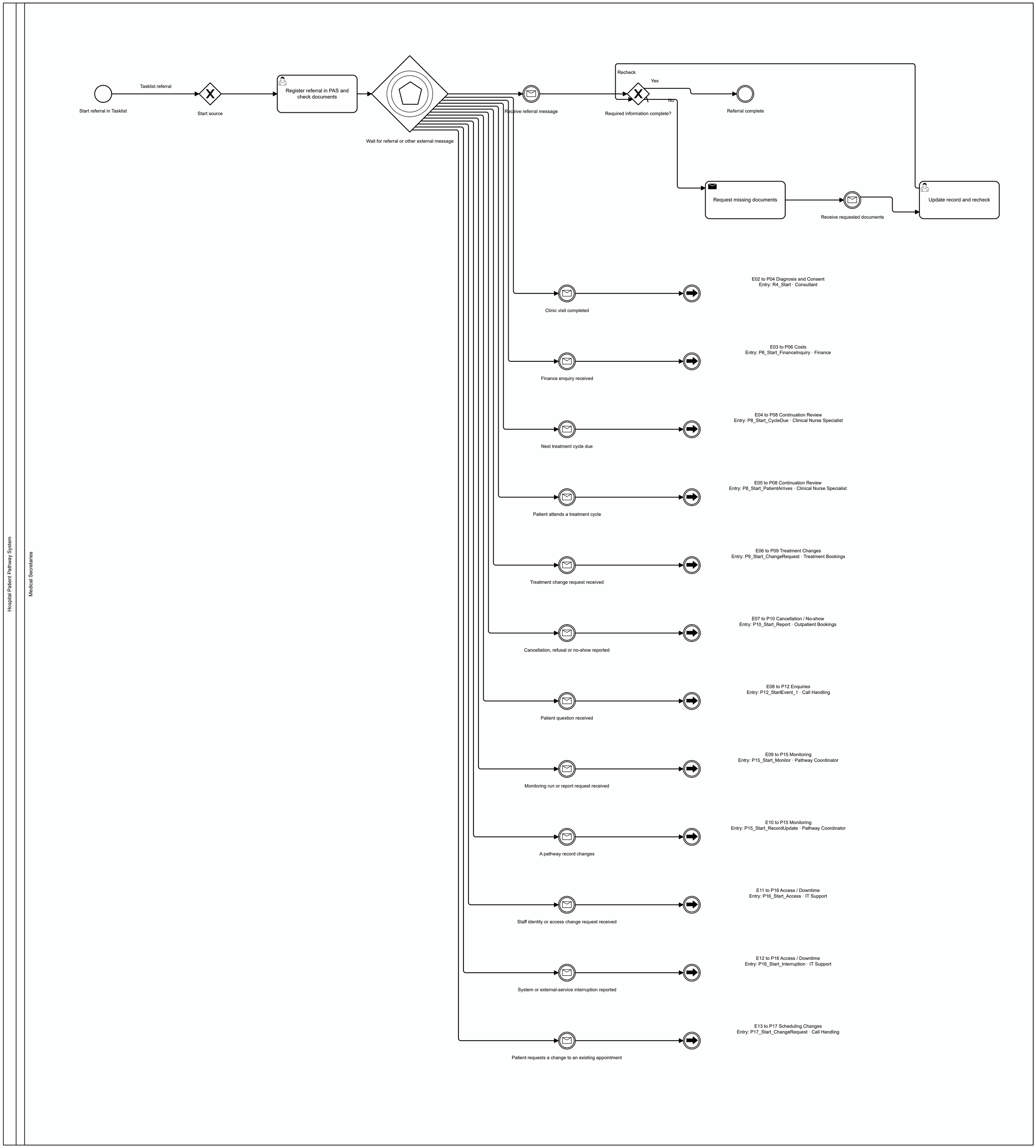
00 – Complete BPMN collaboration

Master Event Pathway — every major event, 11 Sprint Level 3 mapped tasks and messages



01 - Receive Referral

Hospital Patient Pathway | vector export from 1-17.bpmn | zoom to inspect tasks and messages



02 - Review and Referral

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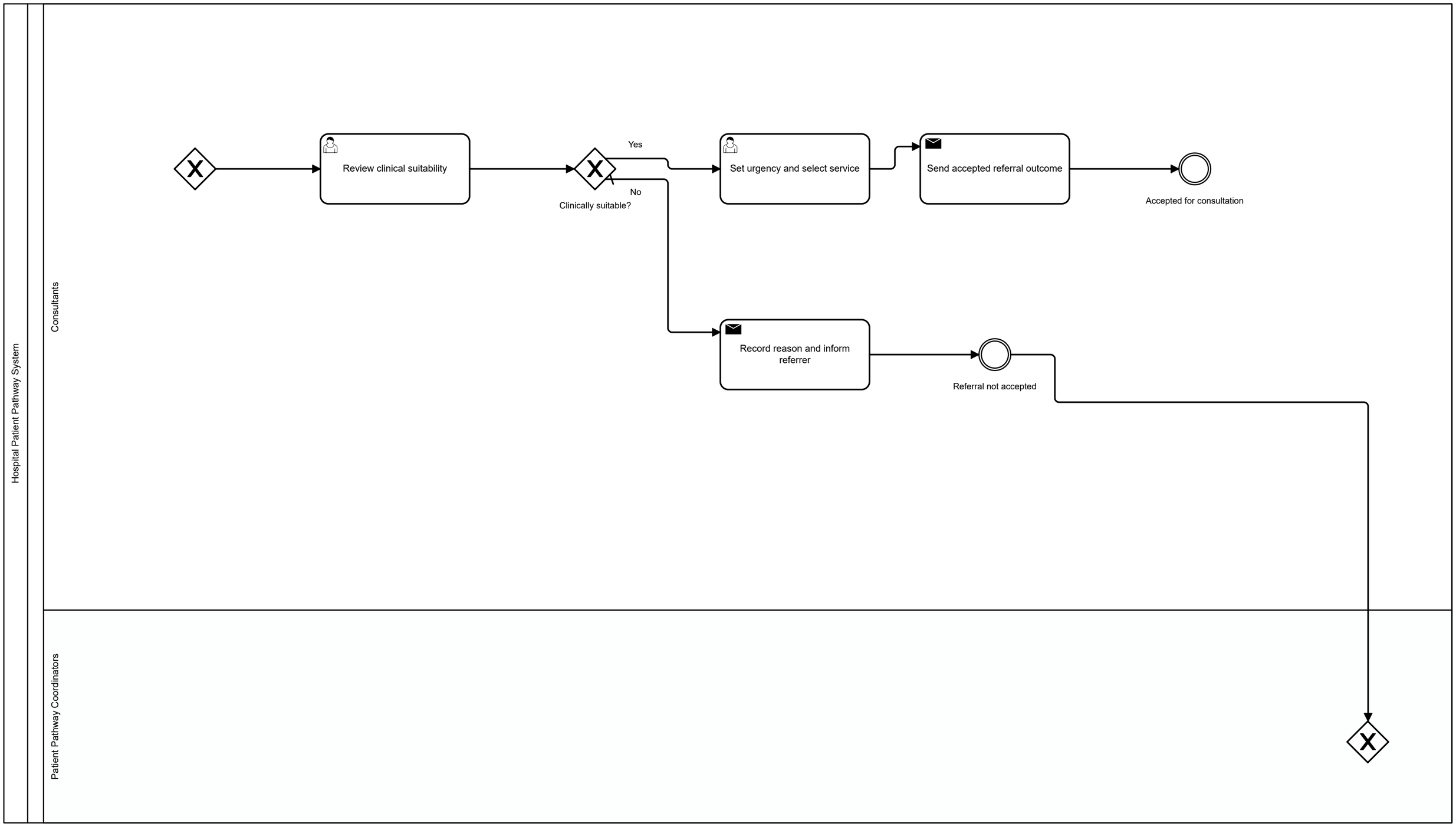

03 - Arrange Initial Consultation and Contact Patient

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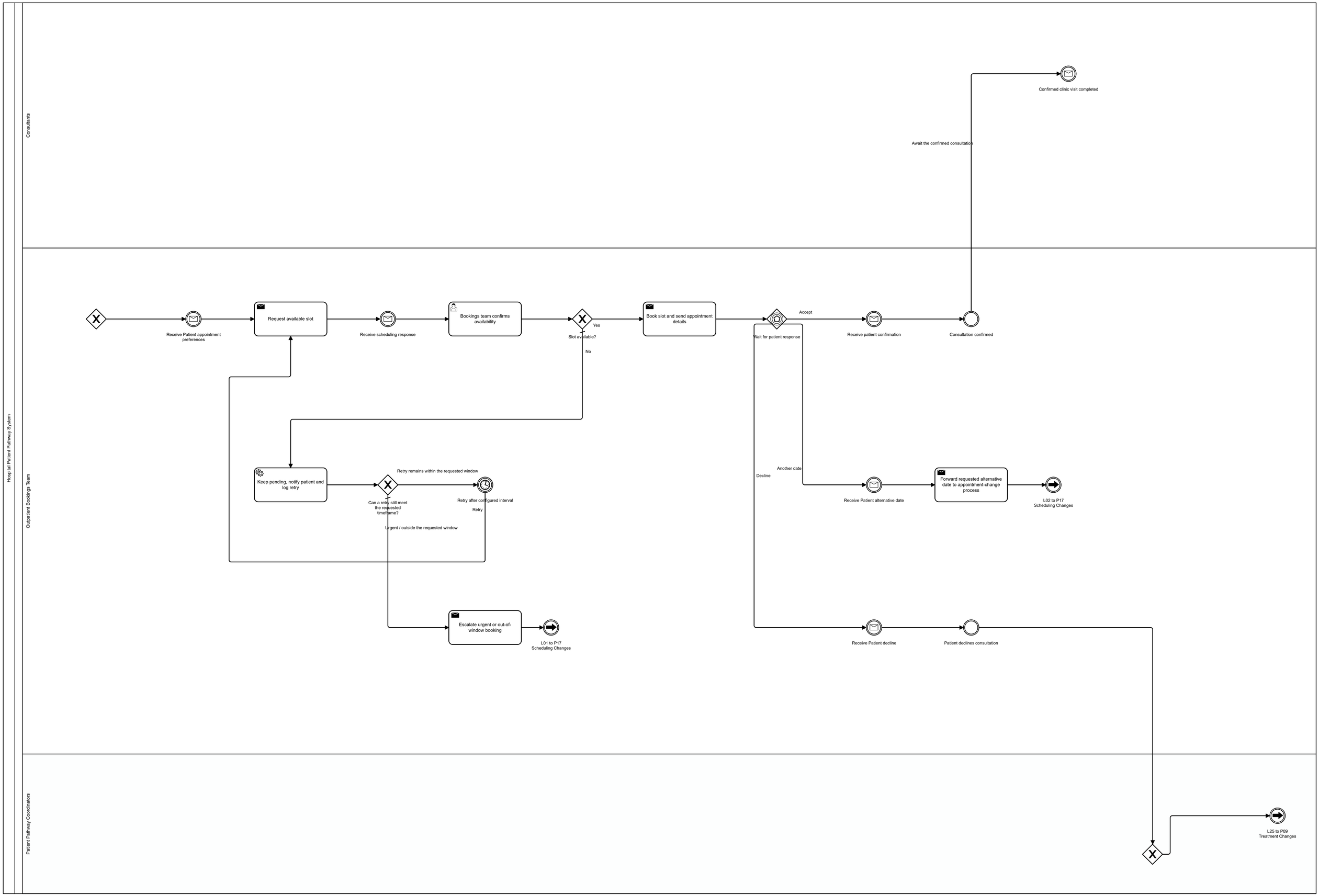

04 - Diagnosis and Obtaining Informed Consent

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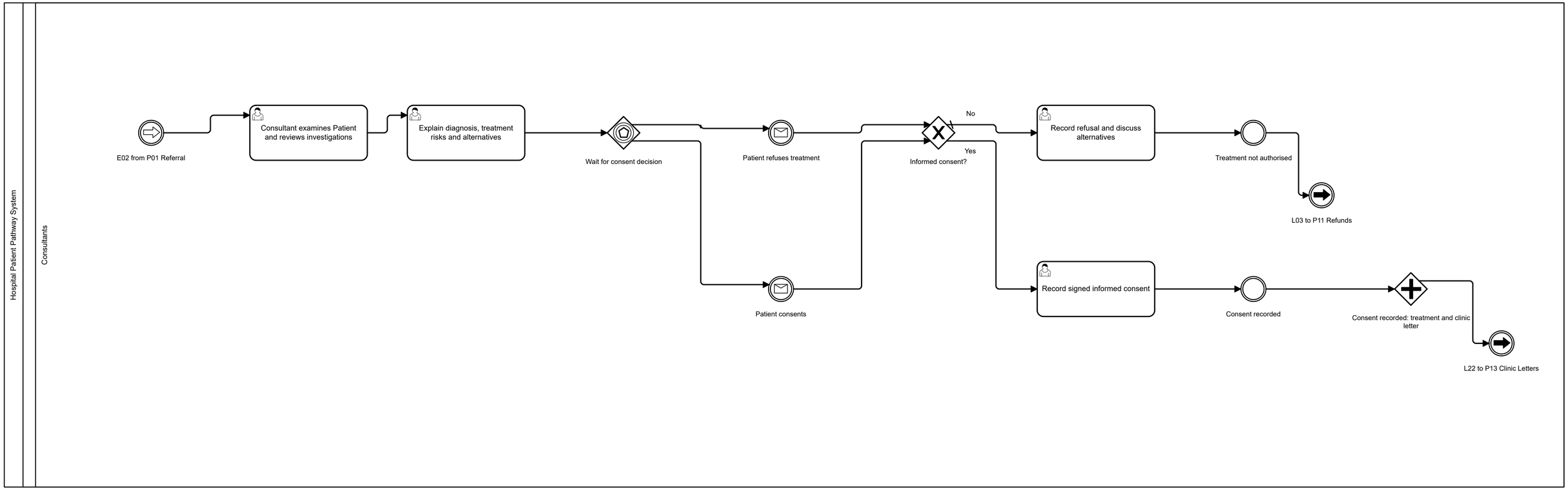

05 - Arrange Treatment and External Services

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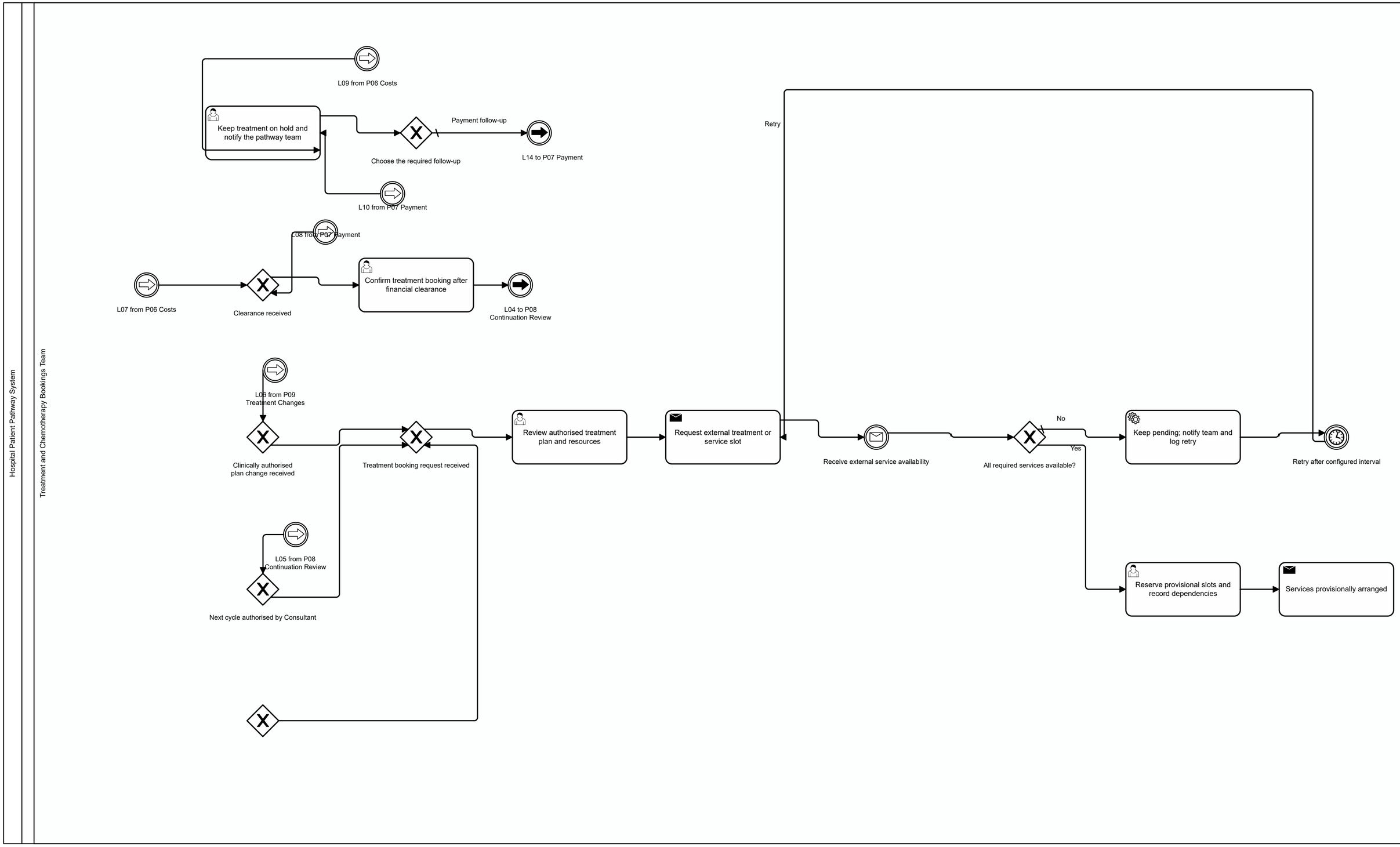

06 - Verify Cost Allocation Method

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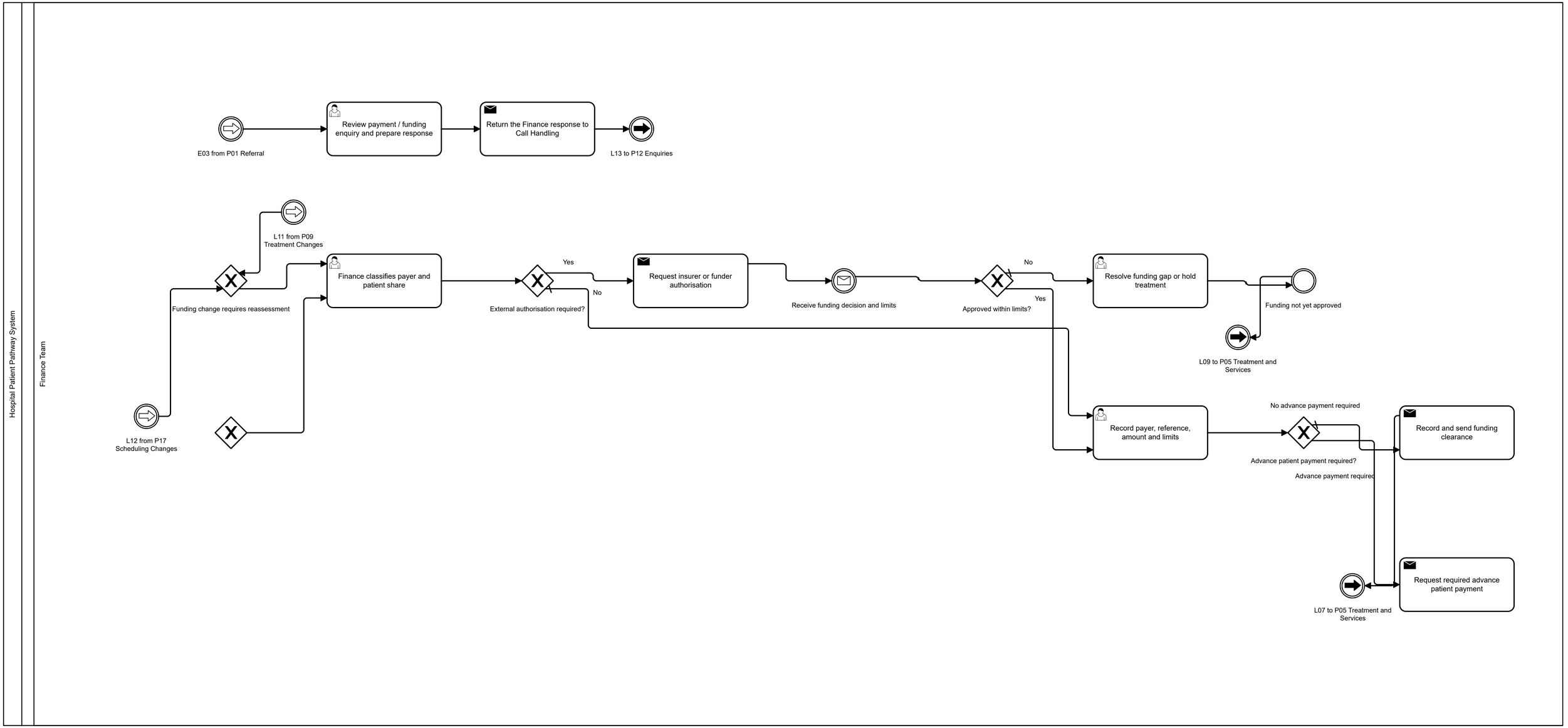

07 - Process Payment Result

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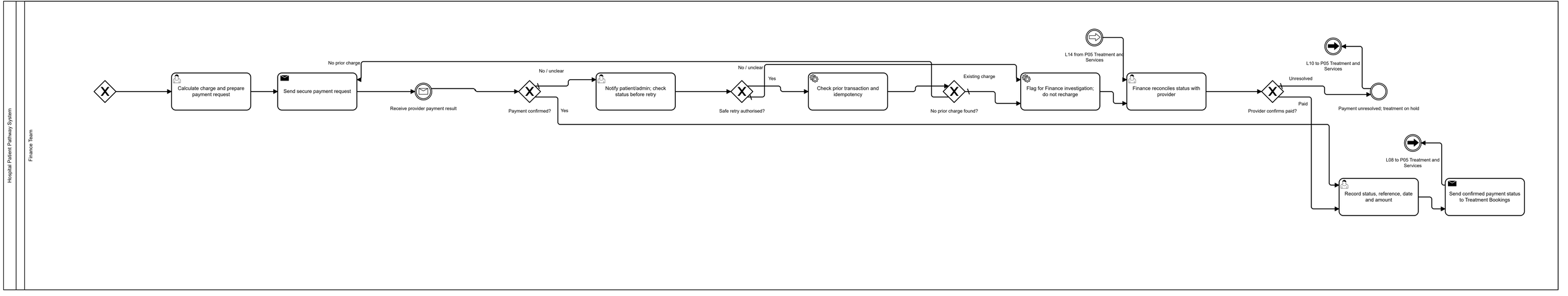

08 - Evaluate Whether to Continue Treatment

Hospital Patient Pathway | vector export from 1-17.bpmn | zoom to inspect tasks and messages

Hospital Patient Pathway System

Consultants

L17 from P17 Scheduling Changes

Scheduling conflict requires clinical prioritisation

Consultant assesses clinical priority for the scheduling conflict

Return the clinical priority decision to the pathway team

L18 to P17 Scheduling Changes

L15 from P10 Cancellation / No-show

Cancellation / no-show needs clinical review

Enter continuation review

L04 from P05 Treatment and Services

Financially cleared treatment booking ready for care

Provide authorised treatment cycle

L21 to P13 Clinic Letters

Clinical review and blood test determine next step

Record clinical findings and assess continuation

Decide whether the patient is fit to continue

Fit to continue?

suitable

not suitable

Go ahead with the next treatment cycle

Send Consultant-authorised next-cycle request to Treatment Bookings

L05 to P05 Treatment and Services

Decide: postpone treatment or change the treatment plan

Postpone or change?

Postpone

Stop treatment

Change treatment plan

Treatment postponed

Wait until clinical reassessment date

Refer treatment stop to Finance for refund review

L20 to P11 Refunds

Send clinically authorised treatment-plan change

Clinical Nurse Specialist Team

Clinical enquiry referred by Call Handling

Qualified clinical team prepares an enquiry response

Return clinical response to Call Handling

Carry out clinical review and take the required blood sample

Test results received

L19 to P12 Enquiries

L17 from P12 Enquiries

E05 from P01 Referral

E04 from P01 Referral

09 - Manage Treatment Changes

Hospital Patient Pathway | vector export from 1-17.bpmn | zoom to inspect tasks and messages

Hospital Patient Pathway System

Consultants

Postpone treatment at once for patient safety

Responsible clinician records and approves the postponement

Confirm formal clinical authorisation for the treatment change

Clinical authorisation confirmed?

Yes

No - request formal authorisation

Formally approved

urgent safety postponement

Treatment and Chemotherapy Bookings Team

E06 from P01 Referral

Internal hand-off or external request

L24 from P17 Scheduling Changes

How should the request be handled?

informal request

Do not process the informal request

Request not processed

Adjust the treatment plan and schedule

Affects an existing charge or funding approval?

no

L06 to P05 Treatment and Services

Change made, no finance review needed

yes

Finance Team

Review the financial implications

L11 to P06 Costs

Patient Pathway Coordinators

L25 from P03 Initial Consultation

10 - Manage Appointment Cancellation, Rejection, and No-shows

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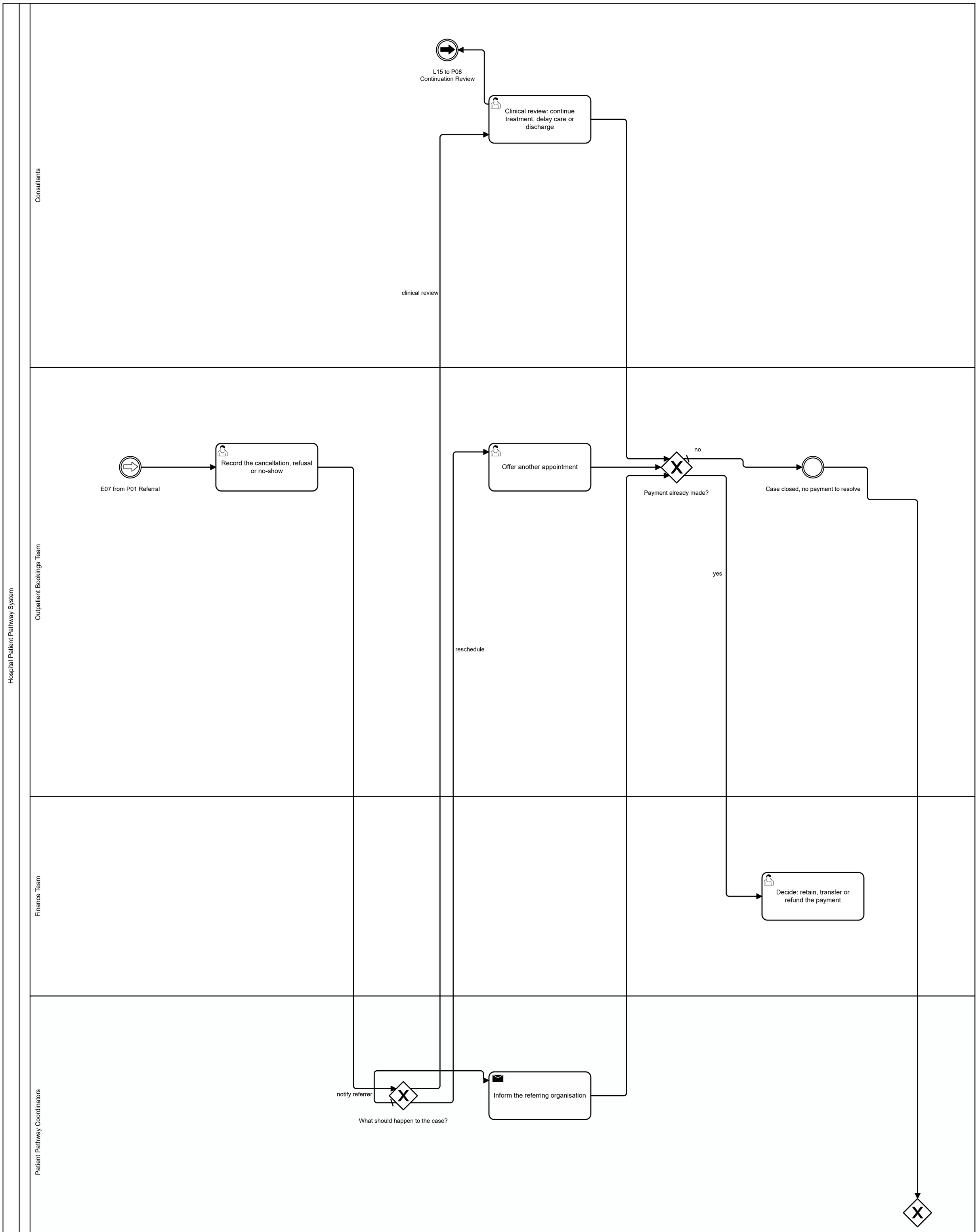

11 - Process Refunds and Fund Transfers

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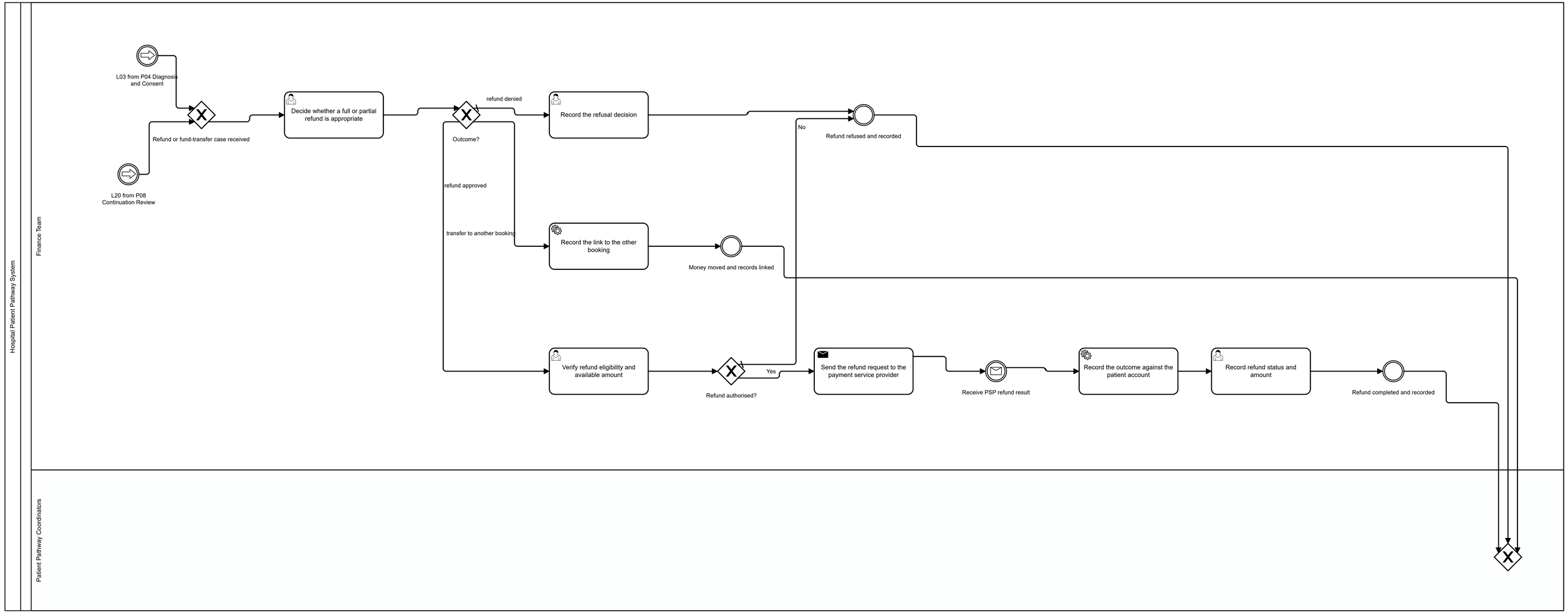

12 - Handle Patient Inquiries

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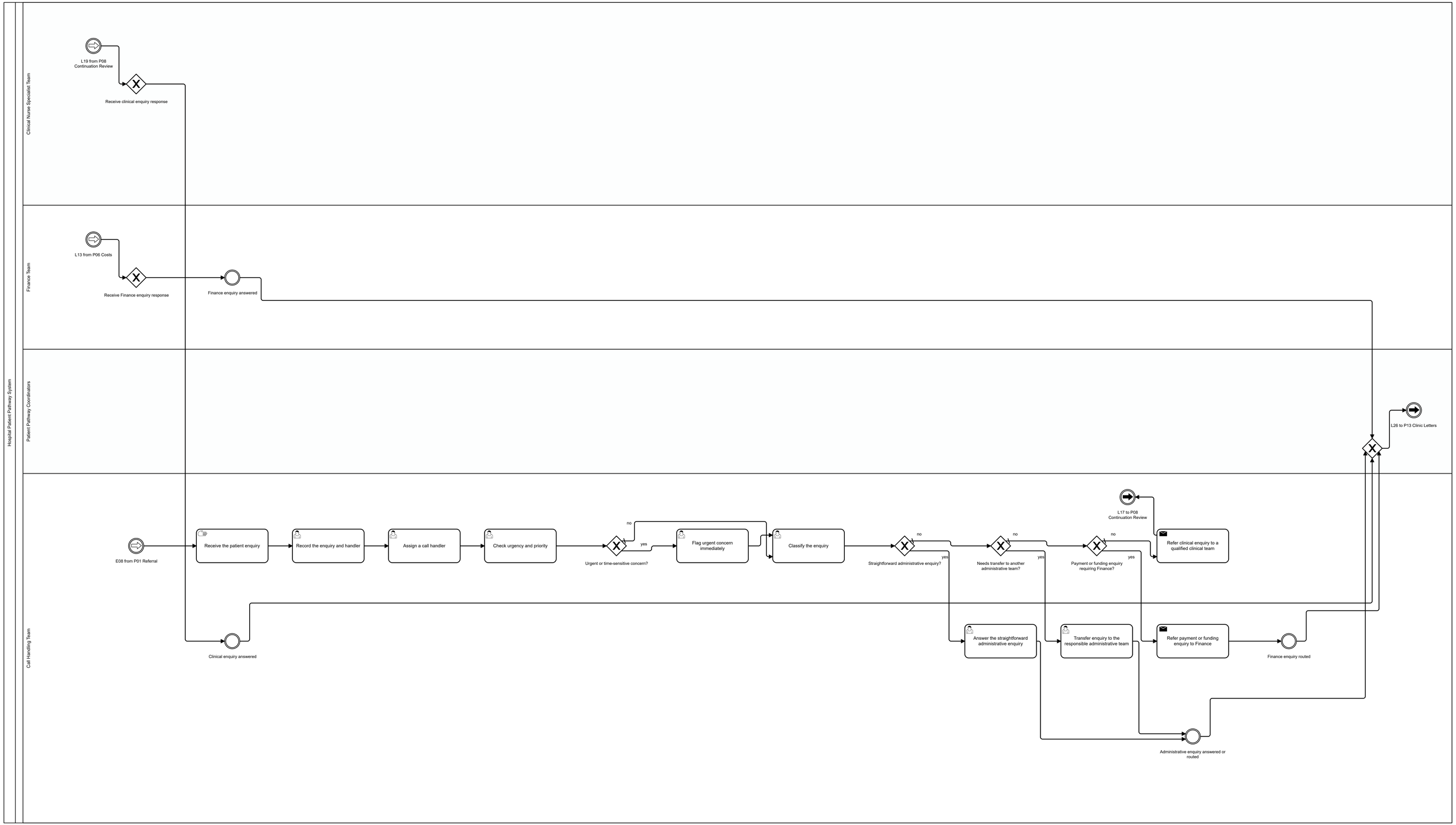

13 - Handle Medical Correspondence

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Hospital Patient Pathway System

Medical Secretaries

Receive approved letter

Check recipients and admin details

No suspected clinical error?

No suspected clinical error

Send approved letter to the correspondence service

Receive letter entry for release

Distribute approved letter

no

Return suspected error to Consultant

Consultants

L21 from P08 Continuation Review

L22 from P04 Diagnosis and Consent

Appointment completed

Open clinic-letter record for this visit

start monitoring clock

Draft the clinic letter

Continue review cycle

no

Review the clinical content

Consultant approves?

yes

Send approved letter to Medical Secretaries

Receive the Consultant response

Consultant response received

Receive corrected letter

Patient Pathway Coordinators

Letter completed or deadline reached?

Deadline

Completed

Wait 7 days

Still outstanding after 7 days?

yes

no

Record overdue letter

Send weekly Consultant reminders

Clinic letter completed

Letter already completed?

Still pending

Already completed

Letter completed or deadline reached?

Deadline

Completed

Wait 23 days

Clinic letter completed

Still outstanding at 1 month?

yes

no

Escalate to Administrative Manager

Letter already completed?

Still pending

Already completed

Letter completed or deadline reached?

Deadline

Completed

Wait 2 months

check status at 3-month threshold

Still outstanding at 3 months?

yes

no

Escalate to senior management

Clinic letter completed

Stop reminders when complete

Retain reminder and escalation history

Monitoring closed

L26 from P12 Enquiries

Administrative Manager

Receive escalation

Contact the Consultant

Consultant contacted by Administrative Manager

Hospital Senior Management

Review escalation and decide action

Senior-management review completed

14 - Arrange Follow-up Appointment

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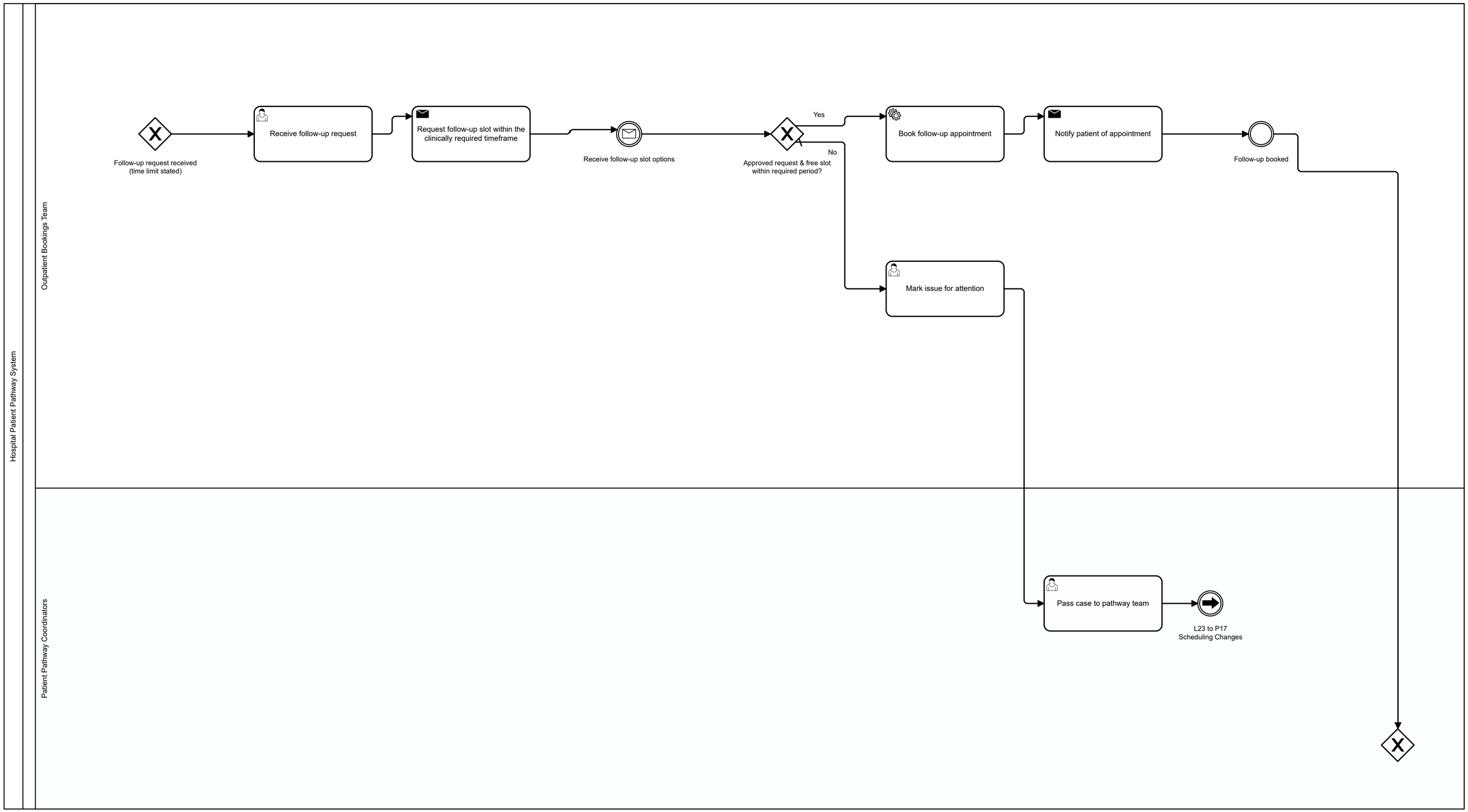

15 - Monitoring Workflow and Report Generation

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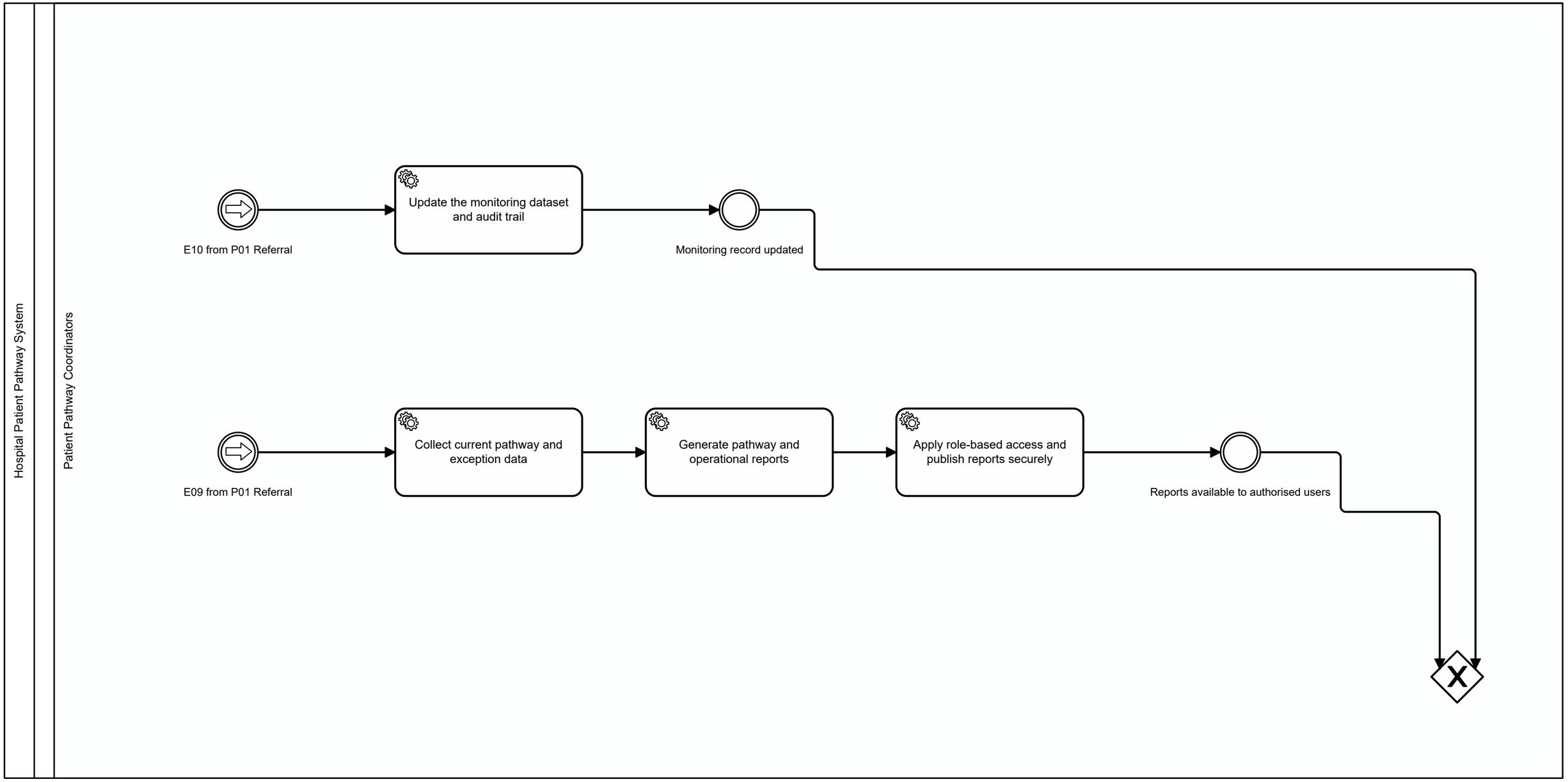

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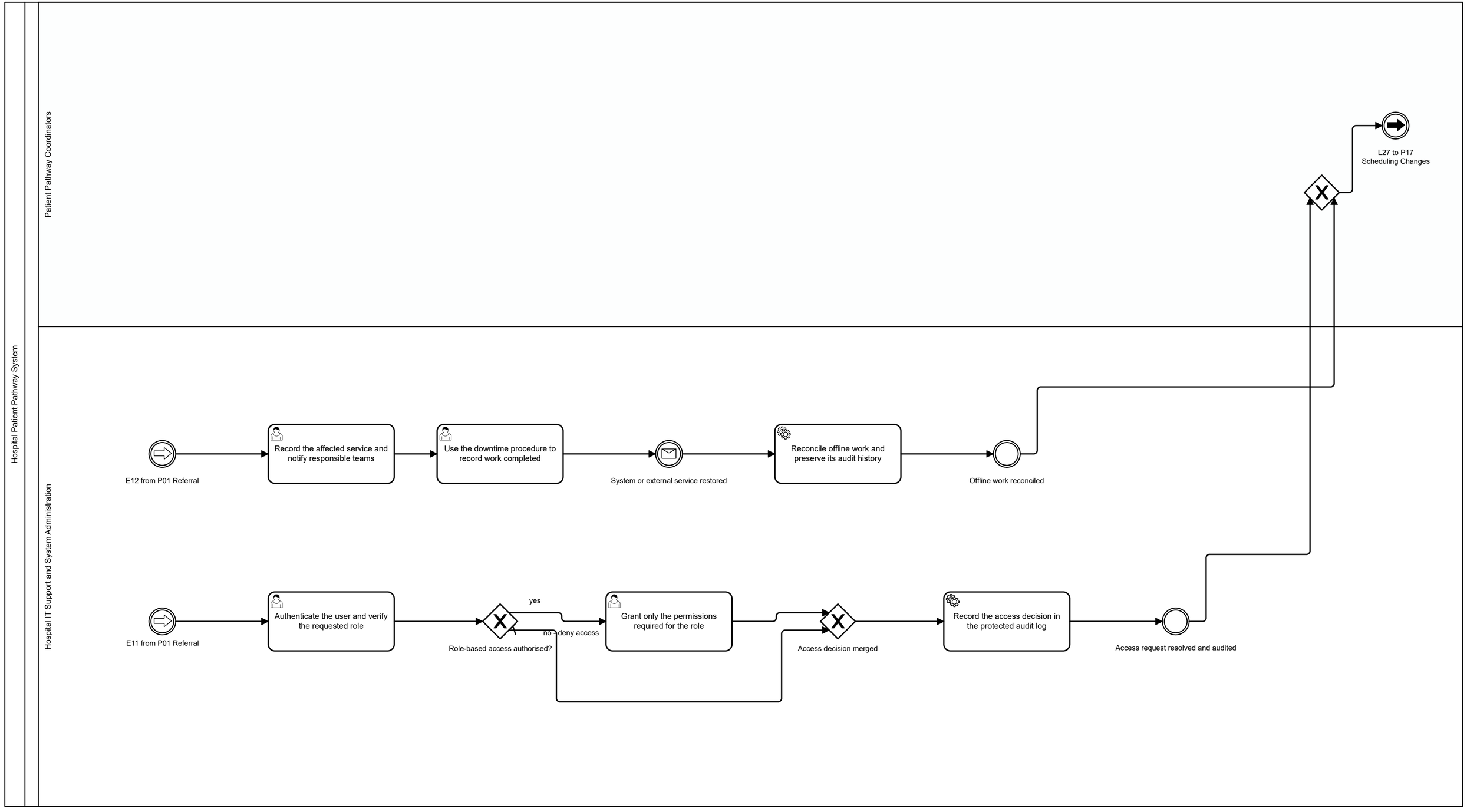
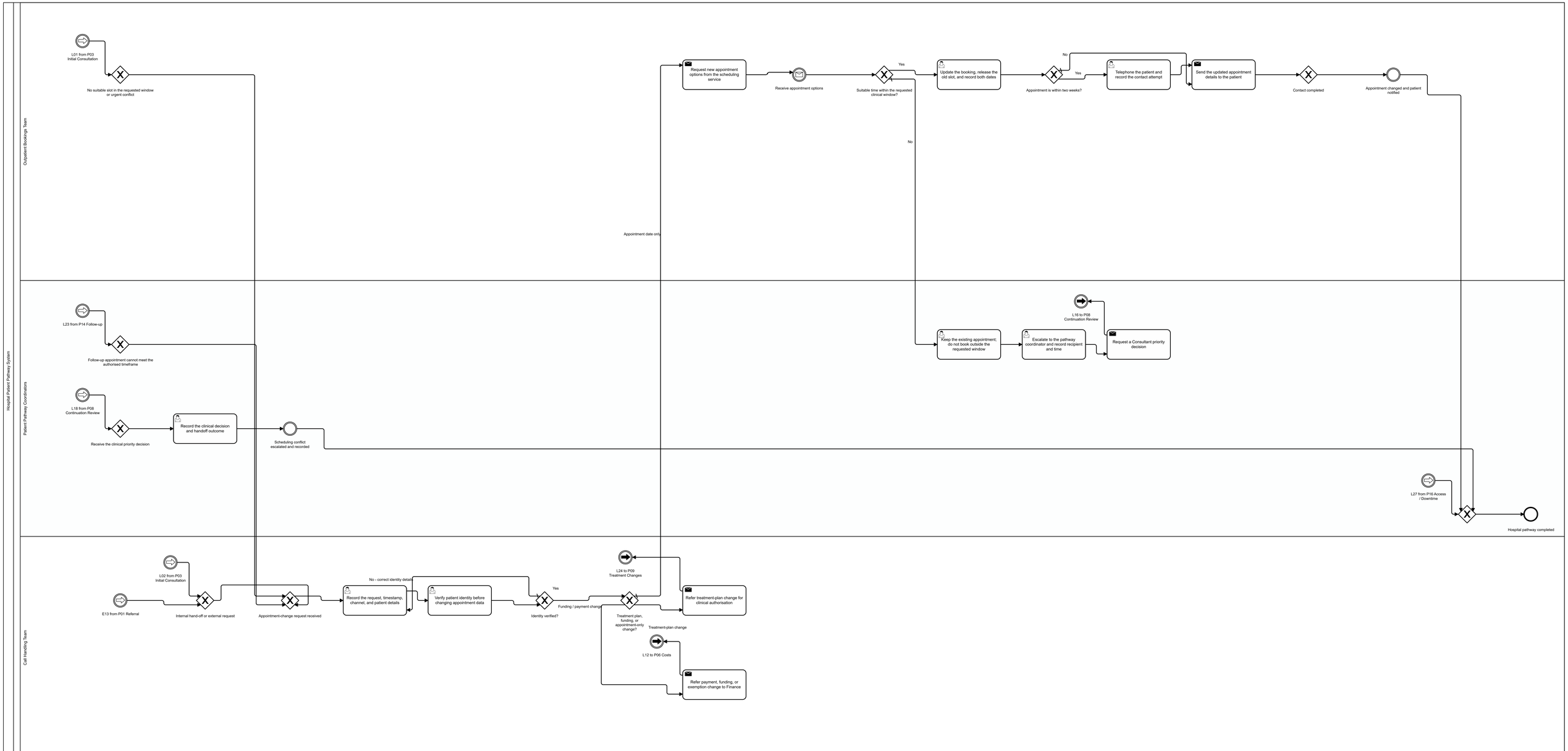


17 - Handle Appointment Change Requests and Escalate Scheduling Conflicts

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